

**Re: Response to 2-15 July 2026
correspondence - incorrect application
of EB12, Award and QH policies - ECC 3
July 2026 (MSH-INJ-5795)**

From: Cory Shepherd

coryshepherd1@hotmail.com

To: LBH Injury Management

LBH_InjuryManagement@health.qld.gov.au,

Lyndelle Forrest

Lyndelle.Forrest@health.qld.gov.au

Cc: Heath Moran heath.moran@together.org.au,

James Zappia james.zappia@art.com.au

Sent: Thursday 30 July at 11:25

Good morning Michelle,

Thank you for your email of 29 July 2026.

This email is sent on Thursday 30 July 2026. My appointment with Dr Ma is Friday 31 July 2026 at 2:00 pm. The Health Service's questions and completed forms (or the same information by email) are required before that time.

THE SEQUENCE

I was working my rostered shifts. Following my request to temporarily reduce my ordinary hours from 0.6 to 0.5 FTE for two weeks, I was told I could not work until I completed an Employee Capability Checklist. I obtained and provided that checklist on 3 July 2026. I was then told I still could not work, pending a review. The requirement has since changed again — to further medical clarification, for which no timeframe has been given.

I was not absent and I was not seeking to recommence. I had been performing the role on reduced hours within my now certified restrictions for approximately twelve

months. I mention this because the questions to Dr Ma should be directed to that situation.

I have elected to take the request to Dr Ma myself, and I have booked an appointment for 2:00 pm on Friday 31 July 2026.

WHAT I NEED BEFORE 2:00 PM FRIDAY 31 JULY 2026

1. The Health Service's questions for Dr Ma, or completed Attachment 1.
2. Completed Attachment 2, or the same information by email.
3. Confirmation of who will be responding before the appointment.
4. Reimbursement of the \$150.00 I have already paid, and the name and address to which the practice should direct the invoice for Friday's consultation.

If the forms are not convenient, the same information in an email is equally welcome — before 2:00 pm Friday.

If the questions are not received in time, I will attend the appointment regardless and ask Dr Ma to address my capacity and restrictions against the duties in the role description (attached). I will forward his response to you. I would prefer to put the Health Service's own questions to him, which is why I ask now.

ABOUT THE ATTACHMENTS

To make this as straightforward as possible, I have attached the following:

Attachment 1 — Duty / Restriction Mapping — sets out the twelve key responsibilities listed in the AO3 role description (also attached). If the Health Service indicates which duty or duties it says I cannot perform, and which restriction is said to prevent it, that identifies precisely what Dr Ma needs to address.

Attachment 2 — Psychosocial Hazard Identification — follows the structure of the Managing the risk of psychosocial hazards at work Code of Practice 2022:

hazard, source in the work, risk, assessment, controls, consultation. The form also provides for the Health Service to confirm the position if no hazard is identified.

Attachment 3 — Invoice 574370 and payment receipt for the \$150.00 case conference of 3 July 2026 (already paid).

Attachment 4 — AO3 Switchboard Services role description (source of the duties in Attachment 1).

Completing and returning Attachment 1 and/or Attachment 2 (or the same information by email) would give me what I need to put to Dr Ma for this step.

1. MY ELECTION

1.1 I will take the request to Dr Ma myself rather than add a further step.

1.2 I do not propose to complete the Employee Authority Form. It is broader than the present request — it extends to disclosures to and from WorkCover Queensland and income protection insurers, and it is open-ended in duration. If a written authority is needed for Dr Ma to respond to a specific request, I am happy to provide one confined to that purpose.

2. COST OF THE CONSULTATIONS

2.1 The consultation of 3 July 2026 was invoiced to me personally — My Doctors Clinic Surfers, invoice 574370, dated 3 July 2026, \$150.00 (item WCO002, case conference). I paid that invoice in full on 10 July 2026 (receipt N471065713633, reference 574370 C Shepherd). The invoice and payment receipt are attached as one document (Attachment 3).

2.2 A further consultation is booked for 2:00 pm on Friday 31 July 2026 and is expected to attract the same fee.

2.3 Both consultations are required by the Health Service for its own assessment; neither is material I

have volunteered. Consistently with the approach taken elsewhere in the public sector, where the entity that requires medical information meets the reasonable costs of obtaining it, I ask:

(a) that the \$150.00 I have paid be reimbursed to me; and

(b) that the Health Service provide the name and address to which the practice should direct Friday's invoice, so that the cost of the Health Service's own request is not again met from my funds.

2.4 If the Health Service does not intend to meet these costs, could you please confirm that in writing, together with the basis for that position.

3. THE QUESTIONS I NEED

3.1 Your email states that "further clarification is required regarding your functional capacity and the medical restrictions identified", but does not identify what is unclear. Dr Ma cannot respond to a request in those terms.

3.2 Could you please provide, in writing, before 2:00 pm Friday:

(a) the specific questions the Health Service asks Dr Ma to answer, set out as questions;

(b) for each question, the duty or task to which it relates — Attachment 1 is provided for this purpose;

(c) what in the 3 July Employee Capability Checklist is said to be insufficient — which entry, and why it does not enable the assessment; and

(d) who will consider Dr Ma's response, and the timeframe within which the decision about my duties will then be made.

4. THE ORIGIN OF THE REQUIREMENT

4.1 Could you please confirm the date on which, and the basis on which, the Health Service required me to complete an Employee Capability Checklist. My understanding is that the requirement was raised in response to my request to temporarily reduce my hours. I am not aware of any incident, complaint or performance concern preceding it.

4.2 Could you also please confirm the outcome of that request to reduce my hours, and if no decision has been made, when one will be made and by whom. EB12 clause 10.3 provides that an employee may ask for a change in the way they work, including their ordinary hours of work; that the request may be refused only on reasonable grounds; and that the employer must give written notice of its decision, with reasons, within 21 days.

5. THE RESTRICTIONS AND THE DUTIES (Attachment 1)

5.1 Since your email refers to the inherent requirements of the role, I have compared the restrictions certified on 3 July with the responsibilities set out in the AO3 role description, which Dr Ma reviewed before completing the checklist. Attachment 1 lists those twelve responsibilities as they appear in that document.

5.2 First, complaint-handling. The ECC of 3 July 2026 records: no complaint-handling duties; complaints received at the Switchboard to be logged and redirected, not actioned or resolved by me. That restriction has already been addressed in correspondence. In her email of 7 July 2026, Ms Forrest stated that the established process is for patient concerns to be redirected to Consumer Liaison Services (CLS); that there is no expectation for Switchboard operators to manage or deal with those matters; and that where a consumer raises concerns about a Switchboard employee, those concerns are referred to the Manager, Switchboard or, in her absence, CLS. My reply of 9 July 2026 noted that this confirmation, together with existing rostering practice, appears to accommodate that ECC recommendation. I treat complaint-handling as closed on the Health

Service's own account of the role. It does not need to be reopened with Dr Ma, and it is not a basis for refusing all duties in Attachment 1.

5.3 Second, the remaining restrictions — the limits on peak weekday day and afternoon shifts, predictable rostering, and a minimum ten-hour break between shifts — go to when I work, not to which duties I perform. They remove none of the responsibilities listed in Attachment 1.

5.4 I note the role description records the position as continuous shift work. I performed that role, on reduced hours and within my certified restrictions, for approximately twelve months, and the Health Service's correspondence of 7 July 2026 records that my restrictions were already being accommodated. If the rostering restrictions are now said to be incompatible with the role, when they were not during that period, could you please identify what has changed.

5.5 Third, reduced ordinary hours are expressly contemplated by the industrial instruments. Clause 8.2 of the Hospital and Health Service General Employees (Queensland Health) Award – State 2015 provides for part-time employment on a regular number of ordinary hours, pro rata to full-time conditions, with a minimum of eight ordinary hours per fortnight. A reduction in hours does not of itself remove any duty from the role. Could you please identify why the reduced hours certified on 3 July are treated differently — that is, what it is about those hours, as distinct from the reasons for them, that prevents the duties of the position from being performed.

5.6 QH-POL-210 applies to all Queensland Health and Hospital and Health Service employees and directs adjustments — including temporary adjustment for rehabilitation and return to work, and for capacity restriction — to enabling an employee to safely perform the genuine occupational requirements of the role. It reflects the Anti-Discrimination Act 1991 (Qld), which contemplates reasonable terms and special conditions where job capacity is restricted by impairment (s 34)

and permits departure only where unjustifiable hardship is established (ss 5, 35). QH-IMP-401-5 makes managers responsible for identifying suitable duties, guided by medical information, and its application is not limited to accepted workers' compensation claims. Each operates at the level of identified duties, which is what Attachment 1 is directed to.

6. THE CHARACTERISATION OF RISK (Attachment 2)

6.1 Your correspondence of 13, 15 and 29 July 2026 refers to whether my return can be "safely" facilitated, whether the matters identified by Dr Ma "present any psychosocial hazards or risks", and to the health and safety of workers "and others in the workplace". I do not suggest the Health Service should not satisfy itself as to safety.

6.2 However, in the twenty-seven days since 3 July, no incident, complaint or hazard has been identified to me. None was raised during the twelve months I performed the role on reduced hours. No finding of any kind has been made in relation to my performance of this role.

6.3 EB12 clause 7.2 and the Code of Practice proceed by identifying a hazard, assessing the risk, and considering controls, in consultation with workers and health and safety representatives — and they identify psychosocial hazards by reference to features of work and its management. Attachment 2 follows that structure.

6.4 Could you please confirm whether it is asserted that my performing my duties within the certified restrictions would present a risk to me or to any other person and, if so, whether the hazard is said to arise from the work and work system or from me, together with the material relied upon. Attachment 2 provides for both. If no such assertion is intended, could you please confirm that in writing.

6.5 For clarity, my cooperation with the medical request does not suspend the other matters: the restoration of

my roster, the treatment of my pay and leave, and the workplace psychosocial assessment remain live in parallel.

7. CONFLICT OF INTEREST

7.1 I raise one further matter, without alleging wrongdoing.

7.2 The requirement that I complete an Employee Capability Checklist was raised by my line manager, Ms Taylor. Ms Taylor is the subject of a complaint which the Ethical Standards Unit determined to be a public interest disclosure, and her conduct is a matter in issue in proceedings before the Queensland Industrial Relations Commission in which she may give evidence.

7.3 The Code of Conduct for the Queensland Public Service provides at clause 1.2 that having a conflict of interest is not unusual and is not wrongdoing in itself, but that failing to disclose and manage it appropriately is likely to be wrongdoing, and that employees will disclose any personal interest that could be seen as influencing the performance of their duties.

7.4 Could you please confirm what involvement Ms Taylor has had in the decisions concerning my attendance, duties and pay since 2 July 2026; whether any conflict of interest has been disclosed and managed, and how; and that decisions concerning my attendance, duties and pay will be made by a person without involvement in those proceedings.

8. SCOPE AND MEETING

8.1 I will provide Dr Ma with the questions and the role description. My consent extends to information concerning my capacity, my restrictions, and their operational application to the duties of my position, and I ask that the questions be limited to what is necessary for that purpose.

8.2 If it would assist, I am willing to meet at short notice — including before Friday — to work through

Attachment 1 duty by duty. I would ask that any meeting occur alongside, rather than in place of, a written response.

9. OTHER MATTERS

9.1 The matters raised in my emails of 28 July 2026 remain outstanding within the period identified in that correspondence. I do not repeat them here, and this email does not replace them.

10. WHAT I ASK

10.1 So that this can be resolved simply, I ask:

(a) that my roster be provided to me and that I be restored to my rostered shifts within the certified restrictions;

(b) that the rostered shifts from 3 July 2026 be processed by ad hoc payment, and the leave debited for that period be re-credited;

(c) that the \$150.00 I have paid for the 3 July consultation be reimbursed; and

(d) that the invoice for Friday's consultation be directed to the Health Service at an address you provide.

10.2 I have facilitated every step asked of me — I obtained the checklist the Health Service required and paid for it myself, I have elected to take its request to Dr Ma myself, I have booked the appointment, and I have prepared the attached forms so that the Health Service's questions can be framed quickly. I am conscious that time and cost are accumulating for both of us, and I would prefer to resolve this now, collaboratively, rather than through further process.

11. IN SUMMARY

11.1 The operative decision in this matter is the direction that I not attend work. Everything since follows from it — the treatment of my rostered shifts, the

refusal of duties in any capacity, and the requirement for further medical information.

11.2 A direction to an employee not to attend work is an exercise of authority. I have asked on more than one occasion for the source of that authority to be identified, and it has not been. Nor have I been told the process under which I am being managed, or the person who made the decision.

11.3 I raise this not as a complaint, but because without it I am unable to understand or respond to the basis on which I am being kept from my substantive position.

11.4 I ask that any further decision concerning my attendance, duties or pay identify the power relied upon, the person making it, and the reasons for it.

I remain certified fit, ready and available to work within the 3 July 2026 Employee Capability Checklist.

Kind regards,

Cory Lea Shepherd

Administration Officer (AO3), Switchboard Services

0417 400 227

MSH-INJ-5795

From: LBH Injury Management

<LBH_InjuryManagement@health.qld.gov.au>

Sent: Wednesday, July 29, 2026 4:10:13 pm

To: Cory Shepherd <coryshepherd1@hotmail.com>;

Lyndelle Forrest <Lyndelle.Forrest@health.qld.gov.au>

Cc: Heath Moran <heath.moran@together.org.au>;

James Zappia <james.zappia@art.com.au>

Subject: RE: Response to 2-15 July 2026

correspondence - incorrect application of EB12, Award and QH policies - ECC 3 July 2026 (MSH-INJ-5795)

Good afternoon Cory,

Thank you for your email dated 28 July 2026. We acknowledge receipt of your correspondence and the attached documentation.

As previously advised, the Injury Management Team requires further information from your treating medical practitioner before we are able to determine whether Metro South Health can safely facilitate your return to duties.

The Employee Capabilities Checklist (ECC) provided has been reviewed; however, further clarification is required regarding your functional capacity and the medical restrictions identified to enable an informed assessment of whether suitable duties can be accommodated safely and operationally.

This request is consistent with Metro South Health's obligations under the *Work Health and Safety Act 2011 (Qld)* to ensure, so far as is reasonably practicable, the health and safety of workers and others in the workplace. While your treating medical practitioner's advice is an important consideration, Queensland Health has an independent obligation to assess the medical information provided alongside the inherent requirements of the role and determine whether duties can be safely facilitated.

To assist in progressing this matter, please confirm whether you would prefer to take our request for further medical information to your treating general practitioner yourself, or alternatively, complete and return the attached Employee Authority Form (EAF) authorising the Injury Management Team to write directly to your treating practitioner. Should you choose the latter option, you will be copied into our correspondence with your treating practitioner for your records.

Once the requested medical clarification has been received, we will review the information and advise you of the outcome regarding your capacity to return to duties.

Kindest Regards

Michelle Harrison

Injury

Management Consultant | LBHS HR Team 1

Human Resources

Logan and Beaudesert Hospital and Health Service

lbh_InjuryManagement@health.qld.gov.au | [Chat on Teams](#)
e. qld.gov.au | [ams](#)
p. 07 3542 2744
a. Mon-Fri 8am - 4pm

Metro South Health recognises and pays respect to the traditional custodians of the land and waters – the Yugambeh, Quandamooka, Jaggera, Ugarapul, Turrbal, and Mununjali peoples – and to Elders, past, present and emerging.



Please be advised that we are currently transitioning into Solv, a new Injury Management Case System. Our documents and communications may change appearance slightly and you may even receive communications from us via the @Solv.com.au email pathway, as this is a new email pathway, please ensure you are regularly checking your Junk folder in your email accounts and save the @solv to your frequent emails so you do not miss any communications. Please note these emails are legitimate and are from us here at lbh_InjuryManagement@health.qld.gov.au. If you have any concerns, please reach out to us via the above contact details.

-----Original Message-----

From: Cory Shepherd <coryshepherd1@hotmail.com>
Sent: Tuesday, 28 July 2026 5:40 PM
To: LBH Injury Management
<LBH_InjuryManagement@health.qld.gov.au>; Lyndelle Forrest <Lyndelle.Forrest@health.qld.gov.au>
Cc: Heath Moran <heath.moran@together.org.au>; James Zappia <james.zappia@art.com.au>
Subject: Response to 2-15 July 2026 correspondence - incorrect application of EB12, Award and QH policies - ECC 3 July 2026 (MSH-INJ-5795)

This email originated from outside Queensland Health. DO NOT click on any links or open attachments unless you recognise the sender and know the content is safe.

Dear Michelle / Injury Management / HR,

I refer to your correspondence and Switchboard management correspondence between 2 and 15 July 2026, including:

- Chloe Taylor, 2 July 2026 — unable to "facilitate" return until a completed ECC; shifts processed as sick leave / sick leave no pay;
- my provision of the completed Employee Capability Checklist (ECC) of 3 July 2026 (Dr Day Hong Ma) while I was rostered and working;
- the same-day direction that I not attend "pending review";
- Injury Management correspondence of 7–13 July 2026 that restricted duties could not be facilitated without further medical clarification from Dr Ma;
- your email of 13 July 2026 to ART that Metro South Health was unable to facilitate my return under the ECC (MSH-INJ-5795 / CLM-317073);
- your email of 15 July 2026 (13:10) — pay sits with QSuper because of a "current" IP claim; alternative duties refused "in any capacity"; WCRA rehabilitation limited to accepted claims; return at employer discretion; further medical about psychosocial risk to my return; and
- leave coding as "income protection no pay" from 13 July 2026.

I remain ready, willing and able to work within the 3 July 2026 ECC. I do not abandon my employment. I am taking industrial advice through Together Union.

I am an HHS administrative employee. The industrial framework that applies is attached. Please respond by reference to each attachment.

ATTACHMENTS

ATT02 — Queensland Public Health Sector Certified Agreement (No. 12) 2025 (EB12) — current agreement

ATT03 — Hospital and Health Service General Employees (Queensland Health) Award – State 2015

ATT05 — Managing the risk of psychosocial hazards at work Code of Practice 2022 (Qld)

ATT09 — QH-IMP-401-5 Workplace Rehabilitation Standard

ATT10 — QH-POL-210 Reasonable Adjustment (HR Policy G3)

ATT12 — Directive 12/24: Special Leave (applies under EB12 cl 9.12)

ATT13 — QH-POL-231 Special leave (HR Policy C7)

ATT14 — Administration Officer, Switchboard Services

(AO3) Role Description

FACTS

The 3 July 2026 ECC certifies me fit for modified duties and reduced hours from 3 July 2026 as a continuation of the existing arrangement. The doctor viewed the AO3 Switchboard position description (ATT14) and recorded that usual switchboard duties remain suitable with stated modifications (including no complaint-handling; limits on peak weekday day/afternoon shifts; predictable rostering; minimum 10-hour break). I was performing work consistent with those limits when I provided the ECC. The same-day direction not to attend disrupted that work. The 28 August 2026 date on the ECC is a review of restrictions — not the start of capacity.

WHAT HAS BEEN WRONGLY APPLIED

1. "Current QSuper Income Protection claim" → ordinary pay sits with ART (your 15 July email)

Wrong application.

- An IP file is not ordinary wages for a permanent employee who is certified fit, ready and available, and directed by the employer not to attend.
- On the material available to me notified by employee (myself) May 2025 and February 2026 and now 3rd July 2026 certification of that partial capacity, ART put benefit payments are not applied from 1 June 2026. Treating the Fund as presently replacing ordinary pay is incorrect on that record. Partial capacity and partial payment does not dislodge this framing.
- IP administration is separate (Mr Zappia). It does not displace ATT02 EB12 cl 6.1 (employment security) or ATT02 EB12 cl 7.1.5(g)–(h) (injury management; management of ill or injured employees).
- Coding "IP no pay" while held out after an ECC is not a correct application of ATT02 or of ATT12/ATT13 (special leave).

2. WCRA rehabilitation only if accepted claim; return purely at "employer discretion" (your 15 July email)

Wrong application.

- Even if WCRA statutory rehab is limited to accepted claims, that does not exhaust your duties under ATT02

(EB12), ATT03 (HHS Award), ATT10 (QH-POL-210) and ATT09 (QH-IMP-401-5).

- ATT10 applies to all Queensland Health / HHS employees and includes temporary reasonable adjustment for rehabilitation/RTW and for capacity restriction — not only accepted workers' compensation claims.
- ATT02 EB12 cl 7.1.5 requires the parties to address injury management, management of ill or injured employees, fatigue, psychosocial issues and the Code of Practice 2022 (ATT05), and security for administrative staff in frontline positions.
- "Discretion" does not correctly authorise indefinite exclusion, refusal of interim duties "in any capacity," and non-payment without applying ATT09, ATT10 and ATT02 Parts 6 and 7.

3. Further medical framing me as the psychosocial risk before return "in any capacity" (your 13–15 July emails)

Wrong application of ATT05 and ATT02.

- ATT05 and ATT02 EB12 cl 7.2 require assessment of psychosocial hazards in the workplace and work system (including workplace risk assessments as requested through an HCF by employer or union).
- The 3 July ECC already identifies workplace stressors (complaint-handling; blame for others' failures; rostering/fatigue; unresolved line-management matters). Those are workplace issues under ATT05 and ATT02 cl 7.1.5(j) and 7.2.
- Framing psychosocial risk as a personal barrier inside Injury Management / QSuper misapplies ATT02 cl 7.2 and cl 7.1.11 (feedback when WHS matters are raised).
- Workplace psychosocial issues I have raised over more than two years are not properly dealt with by indefinite IM hold-out. That process is not the correct forum under ATT02 cl 7.2.
- I will cooperate with reasonable medical requests that are specific and linked to identified job tasks under ATT14. Open-ended further medical is not a correct precondition to all duties where ATT14 and the ECC already define capacity I have been performing.

4. "Unable to facilitate" / no alternative duties (Chloe 2 July; your 13–15 July emails)

Wrong application of ATT09, ATT10 and ATT02.

- Chloe's 2 July email made facilitation conditional on a completed ECC. I provided that ECC on 3 July while

working. Facilitation was then refused and I was directed off — the opposite of facilitating return on the medical basis required.

- The ECC states fit from 3 July as continuation of the existing arrangement and that usual switchboard duties remain suitable with modifications against ATT14. Refusing all duties "in any capacity" does not correctly apply ATT10, ATT09 or ATT02 cl 7.1.5(f)–(h).
- Correct application requires identifying any specific duty and restriction said to be incompatible, what modification was considered, and what was rejected — not a blanket refusal.

5. Leave coding as sick leave / sick leave no pay / "IP no pay" while certified fit and directed not to attend

Wrong application of ATT02, ATT12 and ATT13.

- From 3 July 2026 the relevant absence is employer-directed while I am certified fit and available. That is not ordinary sick leave for that period.
- "IP no pay" is not the correct industrial answer under ATT02 when the employer has directed me off after an ECC.
- ATT02 EB12 cl 9.12 provides that Directive 12/24: Special Leave (ATT12) applies to employees covered by the Agreement. ATT13 QH-POL-231 (C7) also applies.
- Special leave under ATT12 and ATT13 must be applied for employer-directed exclusion while I am ready, willing and able within the ECC. Where the only barrier is refusal to facilitate certified duties, special leave on full pay (or ordinary pay as if at work) must be applied. Please correct coding and identify the delegate and instrument for each day from 3 July.

6. Treating 28 August 2026 as if capacity only starts then

Wrong application of the ECC itself.

- 28 August 2026 is the next review of restrictions. 3 July 2026 is the date fit to commence the certified arrangement. Capacity is certified now.

WHAT YOU MUST APPLY

ATT02 EB12 cl 6.1 — employment security

ATT02 EB12 cl 7.1.5 — injury management; ill or injured employees; fatigue; psychosocial issues + ATT05; admin frontline security

ATT02 EB12 cl 7.2 — psychosocial risk assessments of workplaces

ATT02 EB12 cl 7.1.11 — feedback on WHS matters

ATT02 EB12 cl 9.12 + ATT12 + ATT13 — Special Leave

ATT03 — hours, breaks and rostering with the Agreement

ATT09 — workplace rehabilitation / RTW pathway

ATT10 — reasonable adjustment (including temporary adjustment and capacity restriction)

ATT05 — workplace psychosocial risk

ATT14 — task match to the AO3 role the doctor viewed

REQUESTS — PLEASE RESPOND IN WRITING WITHIN 5 BUSINESS DAYS

1. Facilitate my return under the 3 July 2026 ECC (or a written suitable duties plan implementing it: start date, hours, location, tasks, review date) by applying ATT09, ATT10 and ATT02 cl 7.1.5.
2. Complete a task-by-task match of ATT14 to the 3 July ECC. If you maintain you cannot facilitate, identify each restriction and duty and what was considered and rejected.
3. Be specific about any further medical still said to be required: exact questions; which ATT14 job task each relates to; what on the existing ECC is said to be insufficient.
4. Apply special leave under ATT02 cl 9.12, ATT12 and ATT13 for employer-directed days from 3 July while certified fit and available; correct "IP no pay" / sick no pay coding; identify delegate and instrument.
5. Confirm ordinary pay treatment for periods I was ready and available within the ECC but directed not to attend.
6. Confirm compliance with the direction not to attend is not abandonment or unauthorised absence (ATT02 cl 6.1).
7. Arrange a workplace-directed psychosocial risk assessment under ATT02 cl 7.2 and ATT05.
8. Identify the decision-maker for facilitation, suitable duties, special leave and pay, and the instrument for the 3 July direction not to attend.

I will continue to provide reasonable medical information directed to placement and safety. Please make further requests specific and linked to ATT14 tasks.

My QIRC appeal WC/2024/227 is separate. ART/QSuper is being addressed with Mr Zappia.

I look forward to your written response and a meeting date to facilitate return under the attached agreements and policies.

Yours sincerely,
Cory Lea Shepherd
0417 400 227
MSH-INJ-5795

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